Police Custody

UP-12 | Universal Patient Care

Quick flow

01 Scene Safe + Medical Assessment [EMT / AEMT / Paramedic]

Obtain law-enforcement report • address life threats • determine illness, injury, exposure, and use of force

02 Capacity + Consent [EMT / AEMT / Paramedic]

Custody alone does not remove capacity • capable adults retain the right to accept or refuse care

03 Restraint Safety [EMT / AEMT / Paramedic]

Least restrictive • never prone or hog-tied • protect airway, breathing, circulation, and access to treatment

04 Specific Exposure? [EMT / AEMT / Paramedic]

Chemical irritant: decontaminate • CEW: assess trauma and probe location • agitation: use agitation protocol

05 Transport Strongly Indicated? [EMT / AEMT / Paramedic]

Emergency condition, abnormal findings, significant trauma, concerning exposure, impaired capacity, or chemical restraint

06 Monitor + Reassess [EMT / AEMT / Paramedic]

Serial vital signs • restraint position and circulation • continuous monitoring when agitated or sedated

07 Handoff or Informed Refusal [EMT / AEMT / Paramedic]

Document observations versus reports, force or exposure, restraints, officers, findings, treatment, capacity, and disposition

Overview

- Persons in law-enforcement custody receive the same clinical assessment, treatment, dignity, privacy, and access to emergency care as every other patient.
- Clinical needs determine EMS assessment, treatment, transport recommendation, and destination. Law enforcement retains responsibility for custody and scene security.
- This protocol applies to custody encounters, law-enforcement restraints, chemical-irritant exposure, and conducted-energy weapon exposure. Use the appropriate complaint, trauma, airway, behavioral, or refusal protocol concurrently.

Provider Actions

Custody does not change the standard of medical care.

EMT

- Perform complete BLS assessment and immediate lifesaving care. Treat illness or injury under the applicable protocol.
- Provide chemical-irritant decontamination and remove uncomplicated superficial CEW probes using the approved procedure.
- Assess restraint position, airway, breathing, circulation, distal neurovascular status, and decision-making capacity.

AEMT

- Perform all EMT care. Establish IV/IO access and administer fluids or medications only when indicated by the patient's clinical condition and the applicable protocol.
- Assist with continuous monitoring, serial reassessment, and treatment of respiratory or hemodynamic complications.

Paramedic

- Perform all prior care. Lead evaluation of significant agitation, abnormal vital signs, altered mental status, respiratory compromise, cardiac symptoms, severe injury, or high-risk CEW exposure.
- Provide advanced airway, cardiac, and pharmacologic treatment under the applicable complaint or agitation protocol.

Consent, Capacity & Refusal

Custody alone does not eliminate patient autonomy.

EMT

- Determine whether the adult patient can understand the condition, proposed care, material risks, alternatives, and consequences of refusal.
- A capable adult may refuse assessment, treatment, or transport under the Refusal / Non-Transport protocol. Obtain two vital-sign sets when permitted and document any assessment the patient declines.

AEMT

- Reassess for intoxication, hypoxia, hypoglycemia, head injury, medication effects, psychiatric emergency, or another condition that may impair capacity.

Paramedic

- Evaluate questionable or high-risk capacity and contact Medical Control when uncertainty, disagreement, or a serious suspected condition remains.
- When capacity is absent and an emergency condition exists, provide necessary care and transport under implied consent with law-enforcement assistance when required for safety.

Restraint Safety & Monitoring

Use restraint only for immediate safety or medically necessary care, never for punishment or convenience.

EMT

- Use the least restrictive safe method. Never transport a patient prone, hog-tied, with wrists connected directly or indirectly to ankles, or with pressure on the neck, chest, back, or abdomen.
- Position supine with the head elevated or lateral when clinically appropriate. Do not use a long backboard solely as a restraint device.
- Reassess airway, breathing, circulation, restraint position, skin, and distal neurovascular status at least every 5 minutes and document findings.

AEMT

- Add continuous SpO_2 and serial vital signs for patients restrained because of agitation. Obtain glucose and temperature when clinically indicated.

Paramedic

- Use continuous cardiac monitoring and SpO_2 for significant agitation or chemical restraint. Add waveform $EtCO_2$ after chemical restraint or when significant agitation or respiratory risk is present.
- Midazolam or ketamine may be used per agitation protocol. Continuous airway and cardiac monitoring required.

Chemical-Irritant Exposure

Prevent cross-contamination and provide prompt decontamination.

EMT

- Use appropriate PPE, move the patient to fresh air, remove contaminated clothing, and contain contaminated items when feasible.
- Remove contact lenses when easily accomplished. Irrigate burning or visually impaired eyes with water or normal saline for 10-15 minutes. Wash exposed skin with soap and water without scrubbing.
- Do not apply chemical neutralizers, creams, oils, or eyedrops.

AEMT

- Treat wheezing, bronchospasm, or respiratory distress under the applicable respiratory protocol and obtain vascular access when clinically indicated.

Paramedic

- Manage persistent respiratory, cardiac, ophthalmic, or neurologic complications and determine need for specialty consultation or destination.

Conducted-Energy Weapon Exposure

Assess the patient and associated trauma before addressing probes.

EMT

- Confirm the device has been rendered safe by law enforcement. Assess for fall, head, neck, penetrating, and other traumatic injury before probe removal.
- Remove only an uncomplicated superficial probe in soft tissue using the approved procedure. Clean and dress the wound and dispose of the probe as a contaminated sharp.
- Leave probes involving the eye or face, neck, genitalia, breast, joint, bone, or suspected major vessel in place; stabilize and transport.

AEMT

- Perform all EMT care and treat associated injury, dehydration, or other clinical findings under the applicable protocol.

Paramedic

- Obtain ECG and provide complaint-specific evaluation when the patient has chest pain, dyspnea, abnormal vital signs, altered mental status, significant intoxication or agitation, prolonged or repeated exposure, pregnancy, significant trauma, or another concerning finding.
- An awake, alert, asymptomatic patient with normal findings after a brief exposure does not require routine ECG, laboratory testing, prolonged observation, or automatic transport solely because of the electrical exposure.

Indications

- Any patient detained, arrested, incarcerated, or physically restrained by law enforcement.
- Any patient evaluated after chemical-irritant spray, conducted-energy weapon exposure, or force used during apprehension or custody.
- Any request for EMS assessment before admission to a jail or detention facility.

Assessment | Medical assessment

- Obtain the patient's complaint directly whenever possible and obtain a separate law-enforcement report describing the event, use of force, chemical exposure, CEW exposure duration or number of cycles, fall, struggle, and observed behavior.
- Perform a primary assessment, focused medical and trauma examination, and at least two complete vital-sign sets when feasible.
- Evaluate for hypoxia, hypoglycemia, hyperthermia, head injury, neck compression, intoxication or withdrawal, overdose, psychiatric emergency, severe agitation, respiratory disease, and cardiac symptoms.
- Do not attribute abnormal behavior solely to intoxication, psychiatric illness, resistance, or custody status.

Assessment | Restraint assessment

- Ensure restraints do not compromise airway access, ventilation, circulation, neurologic assessment, patient positioning, or treatment.
- EMS personnel will not apply handcuffs or attach locking law-enforcement restraints to the stretcher or side rails.

- When locking law-enforcement restraints remain during transport, a law-enforcement officer capable of immediately releasing them and possessing the key must accompany the patient in the ambulance.
- Request immediate adjustment or removal of any restraint that interferes with necessary medical assessment or care.

Assessment | Transport indicators

- Transport for an emergency condition, abnormal vital signs, altered mental status or impaired capacity, significant force or trauma, head injury, neck compression, hyperthermia, suspected ingestion, respiratory or cardiac symptoms, persistent chemical-irritant symptoms, high-risk CEW findings, severe agitation, or chemical restraint.
- Persistent eye pain, abnormal vision, corneal concern, wheezing, dyspnea, chest pain, syncope, focal neurologic findings, or worsening symptoms require transport.
- Do not delay care or transport to complete law-enforcement questioning, evidence collection, charging, or confinement paperwork.

Assessment | Required custody documentation

- Document custody status; chief complaint; patient statements; capacity assessment; vital signs; examination; treatment; response; and disposition.
- Document the reported mechanism, force, restraint, chemical agent, CEW mode, number or duration of cycles when known, falls, and probe locations.
- Record restraint type, position, application time when known, serial airway and circulation checks, adjustments, and the officer accompanying the patient with immediate access to the key.
- Record names or identifying numbers of involved officers when available, receiving personnel, refusal discussion, signatures, Medical Control contact, and any assessment or treatment the patient declined.
- Clearly identify which information EMS directly observed and which information was reported by the patient, law enforcement, or witnesses.

Treatment steps

- Treat immediate life threats and follow the applicable complaint-specific, trauma, airway, cardiac, toxicology, behavioral, pediatric, or obstetric protocol.
- Clinical needs determine the EMS transport recommendation and destination. Law enforcement retains responsibility for custody and scene security.
- Do not use this protocol as a fit-for-confinement examination. When a potentially serious injury or emergency condition exists, transport for appropriate medical evaluation.
- A capable adult who declines care may remain in law-enforcement custody only after the standard informed-refusal process is completed.
- A patient restrained because of agitation requires continuous direct observation during transport and reassessment after every intervention or change in condition.
- Release after chemical-irritant or brief CEW exposure is appropriate only when the patient is alert, has decision-making capacity, has normal vital signs and examination, has no transport indicator, and completes the applicable refusal or non-transport documentation.
- Provide an early receiving-facility report describing custody status, restraints, significant force, exposure, findings, treatment, and safety considerations.

Medications

Midazolam

Per Agitation protocol

- Paramedic only for agitation when indicated.
- Continuous airway and cardiac monitoring required.

Ketamine

Per Agitation protocol

- Paramedic only for agitation when indicated.
- Continuous airway and cardiac monitoring required.

Warnings / precautions

- Transport is strongly indicated for the listed high-risk features; those criteria alone do not establish incapacity or authority for involuntary transport. If a legally authorized decision-maker refuses, assess and document decision-specific capacity, explain material risks and alternatives, contact Medical Control for high-risk refusal, and follow the approved refusal/safeguarding procedure. Use an authorized emergency or protective-custody pathway when its legal conditions apply. A patient aged 16 or 17 remains a minor for consent purposes unless a legally recognized exception applies; the clinical age-16 dosing boundary does not establish legal adulthood.
- Never use EMS restraints, equipment, medications, or procedures solely for punishment, law-enforcement convenience, interrogation, or evidence collection.
- Never transport a patient prone, hog-tied, sandwiched between backboards or mattresses, or positioned in a manner that compromises airway, breathing, circulation, or access to care.
- Do not place a surgical mask, oxygen mask, or spit-protection device in a manner that obstructs the airway, impairs ventilation, or prevents continuous observation.
- A badge, handcuffs, intoxication, or apparent behavioral disturbance does not exclude serious illness or injury.

Clinical pearls

- Use calm communication, explain each medical action, preserve dignity, and minimize the number of personnel speaking to an agitated patient.
- Distinguish in the PCR between findings directly observed by EMS and events reported by the patient, officers, or witnesses.
- Simple brief CEW exposure in an otherwise normal patient rarely causes a clinically important electrical injury; associated trauma, struggle, intoxication, hyperthermia, and agitation often determine risk.
- Chemical-irritant symptoms usually improve after removal from exposure and decontamination, but persistent ocular or respiratory symptoms require further evaluation.

Special populations | Pediatrics

- Use pediatric protocols for patients younger than 16 years. A parent or legal guardian ordinarily provides consent or refusal unless another legal exception applies; custody status does not change pediatric clinical care.

Special populations | Pregnancy

- For a known pregnant inmate in correctional custody, use the least restrictive restraint and request removal whenever the restraint interferes with medical assessment, treatment, positioning, or transport.
- Do not restrain the patient's hands behind her back. During labor or delivery, do not use restraints around the ankles, legs, or waist.
- Under Tennessee Code § 41-51-202, a healthcare professional responsible for the inmate's health and safety may request that correctional restraints not be used or be removed.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - SOP 604 Patient Refusal and SOP 704 Physical Restraint. | Tennessee Public Chapter 751, effective July 1, 2026 - medical attention for arrestees before jail acceptance. | Tennessee Code § 41-51-202 - use of restraints on a pregnant inmate. | National Association of EMS Physicians, International Association of Fire Chiefs, and International Association of Chiefs of Police - Best Practices for Collaboration During Acute Behavioral Emergencies, 2024. | American Academy of Emergency Medicine - Evaluation After Conducted Energy Weapon Activation. | Centers for Disease Control and Prevention - Riot Control Agents and Chemical Decontamination. | North Carolina College of Emergency Physicians UP-12 Police Custody source protocol retained for historical comparison.

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